

1. Answer: B. A state of well-being where a person can realize his own abilities can cope with normal stresses of life and work productively.

Reason: This is the World Health Organization's (WHO) classic and comprehensive definition of mental health. Option A is more specific to reality testing, C is the definition of psychiatric nursing, and D is a negative definition (absence of illness) which is incomplete.

2. Answer: A. Administers medications to a schizophrenic patient.

Reason: The role of the nurse as a technician involves performing specific, manual, or procedural tasks that require technical competence, such as administering medications, performing treatments, or monitoring vital signs. Option B (feeding/bathing) is the role of a caretaker or caregiver, C is the coordinator role, and D is the educator role.

3. Answer: A. Subconscious.

Reason: In Freud's topographical model, the subconscious (preconscious) is the level of awareness that contains memories and information that are not currently in conscious awareness but can be easily retrieved with effort. "Give me 10 minutes to recall" indicates the information is stored but not immediately accessible, which is the preconscious.

4. Answer: D. The censoring portion of the mind.

Reason: The superego is the moralistic and idealistic part of the personality. It represents internalized societal rules, values, and parental standards. It acts as a censor or conscience, judging our actions and thoughts and producing feelings of guilt or pride.

5. Answer: D. Case finding and surveillance in the community.

Reason: Primary prevention aims to prevent the occurrence of a mental disorder in a vulnerable population. Case finding, surveillance, and health education in the community are examples of primary prevention. Option A (resuming self-care) is tertiary prevention, B (ensuring safety of suicidal client) is secondary/tertiary, and C (stress management) is primary but D is a broader and better example of community-level primary prevention.

6. Answer: B. Conflict between Id and Ego.

Reason: According to Freud, anxiety arises when the ego perceives a threat from the id's impulses (which are unacceptable) and the superego's demands. This internal conflict signals a danger that the ego must handle, often by using defense mechanisms.

7. Answer: B. pre-school.

Reason: According to Erikson, the stage of Initiative vs. Guilt occurs during the preschool years (ages 3-5). During this stage, children begin to assert their power and control over the world by planning activities, creating games, and initiating tasks. Success in this stage leads to a sense of purpose.

8. Answer: C. genuineness.

Reason: Congruence (or genuineness) in nursing means the nurse's internal feelings and external expressions are aligned. The nurse is authentic, real, and honest with the client. Trust, respect, and empathy are distinct therapeutic conditions but do not specifically define the congruence described.

9. Answer: C. Anger.

Reason: The client is expressing anger towards God ("How could God do this to me?"). According to Kübler-Ross's stages of grief, this is

characteristic of the anger stage, where the individual lashes out and asks "Why me?"

10. Answer: D. "It must really be frustrating for you. How can I best help you?"

Reason: This is the most therapeutic response. It validates the client's feelings of frustration and offers support and a choice to the client. It is open-ended, empathetic, and non-judgmental. Option A is a form of passing the client off, B is reinforcing the feeling without exploring it, and C is judgmental.

11. Answer: D. Giving broad opening.

Reason: "What are you thinking about?" is a broad, open-ended question that encourages the client to explore and verbalize their thoughts and feelings without leading them in a specific direction. Focusing (A) would narrow the topic; validating (B) would check the nurse's perception; reflecting (C) would redirect the client's own statement back to them.

12. Answer: B. Suicidal ideation.

Reason: A client entrusting a beloved possession (like a wedding ring) to the nurse and asking for prayers can be a significant signal of impending suicidal behavior. It often indicates the client is "giving away" their prized possessions and preparing for death, which is a serious warning sign.

13. Answer: A. Remove all potentially harmful items from the client's room.

Reason: While all options are important in the care of a suicidal client, the priority intervention is to ensure a safe environment by removing means of self-harm (e.g., sharp objects, belts, shoelaces, glass). This is a direct environmental safety measure that must be implemented first and foremost.

14. Answer: a. Provide as much structure as possible for the child

Reason: Children with ADHD benefit significantly from a structured environment with clear, consistent routines and expectations. This helps them manage their impulsivity and inattention.

15. Answer: d. Serious violations of age-related norms

Reason: Conduct disorder is characterized by a repetitive and persistent pattern of behavior that violates the basic rights of others or major age-appropriate societal norms or rules. This includes aggression, destruction of property, deceitfulness, and theft.

16. Answer: a. Increased attention span and concentration

Reason: This is the therapeutic effect of Ritalin (methylphenidate), not a side effect. Common side effects include decreased appetite, insomnia, and tachycardia. Options b, c, and d are incorrect side effects.

17. Answer: a. Returning the child to the school immediately with family support

Reason: The primary treatment for school phobia (separation anxiety disorder) is to get the child back to school as quickly as possible, with the support of the family and school. This prevents the anxiety from becoming entrenched and avoids secondary gain from staying home.

18. Answer: c. Moderate

Reason: The classifications of Mental Retardation (Intellectual Disability) by IQ are:

Mild: 50-55 to approximately 70

Moderate: 35-40 to 50-55

Severe: 20-25 to 35-40

Profound: Below 20-25

An IQ of 45 falls within the moderate range.

19. Answer: a. Overprotection of the child

Reason: Overprotection is discouraged as it can hinder the child's development of independence and self-esteem. The other options (patience, realistic goals, and reasonable compliments) are all therapeutic guidelines.

20. Answer: b. Altered parenting role

Reason: The parents' expressed apprehension about their ability to care for their child directly relates to their perception of their role and competence as parents.

21. Answer: b. Intolerance to change, disturbed relatedness, stereotypes

Reason: These are classic features of Autism Spectrum Disorder. Children with autism often have a strong need for sameness, difficulty with social interaction (disturbed relatedness), and engage in repetitive, stereotyped movements or behaviors.

22. Answer: d. Rearrange the environment to activate the child

Reason: A therapeutic environment for an autistic child should be stable, predictable, and safe. Rearranging the environment would be distressing and counterproductive due to their intolerance to change. Providing acceptance, safety, and diversionary activities are appropriate.

23. Answer: c. Pre-operational

Reason: According to Piaget, the pre-operational stage occurs from approximately ages 2 to 7. Children in this stage are developing language and symbolic thinking but are still egocentric and lack the logic of concrete operations.

24. Answer: a. Apraxia

Reason: Apraxia is the inability to perform a learned, purposeful motor act despite having the physical ability and desire to do so. Using a toothbrush to comb hair is a loss of the ability to use the object correctly (ideational apraxia). Agnosia is failure to recognize objects; Aphasia is language impairment; Amnesia is memory loss.

25. Answer: c. "This must be difficult for you and your mother."

Reason: This response validates the daughter's feelings and demonstrates empathy. It is a therapeutic, non-judgmental response that acknowledges the difficulty of the situation.

26. Answer: c. Remains in a safe and secure environment

Reason: In moderate-stage dementia, the client's safety is the highest priority due to risks of wandering, falls, and poor judgment. While nutrition and self-care are important, safety is the primary intervention.

27. Answer: d. "What made you say that your husband is alive?"

Reason: This is an example of exploring the client's perception or "what made you say that?" It is a gentle way to understand the client's reality without arguing or confronting the client's delusion (which is more therapeutic than bluntly stating the truth as in options A and B).

28. Answer: b. Insidious onset

Reason: Dementia has a gradual, chronic, and typically irreversible onset. Delirium is characterized by an acute, rapid onset, clouding of consciousness, and fluctuating level of consciousness.

29. Answer: a. Set realistic limits to the client's behavior

Reason: In the initial phase of caring for a manic client, the priority is to establish boundaries and set clear, firm, and realistic limits on demanding, manipulative, or hyperactive behavior to ensure safety and structure.

30. Answer: d. Cleaning

Reason: In the acute phase of mania, activities should be simple, gross motor, non-competitive, and have a clear endpoint. Cleaning is a good task that can help channel the client's excessive energy. Table tennis and chess may be too competitive and overstimulating. Painting requires more fine motor control and attention span than a manic client may possess.

31. Answer: a. Agree on a consistent approach among the staff assigned to the client.

Reason: A consistent approach by all staff is essential for a therapeutic milieu, especially with manipulative clients. This prevents the client from splitting staff or pitting one staff member against another.

32. Answer: a. Taking a directive role in verbalizing feelings

Reason: When a client's anger is escalating, the nurse should take a calm, directive role to help the client verbalize feelings and regain control. Seclusion and restraints should be used as a last resort, not as a first intervention.

33. Answer: d. Hold the next dose and obtain an order for a stat serum lithium level

Reason: Diarrhea and vomiting can lead to dehydration, which increases the risk of lithium toxicity. The priority is to hold the dose and check the serum lithium level to prevent toxicity.

34. Answer: c. Manifest enduring patterns of inflexible behavior

Reason: The hallmark of a personality disorder is an enduring, inflexible, and pervasive pattern of inner experience and behavior that deviates from cultural expectations. These patterns are stable and of long duration.

35. Answer: d. Antisocial

Reason: A lack of remorse and disregard for the rights of others (including abusive behaviors) are defining characteristics of Antisocial Personality Disorder.

36. Answer: b. Reaction formation

Reason: Reaction formation is a defense mechanism where an individual behaves in a way that is the exact opposite of their unconscious, unacceptable feelings or impulses. By preaching against abuse, the client may be unconsciously defending against their own aggressive or abusive impulses.

37. Answer: a. Lack of self esteem, strong dependency needs and impulsive behavior

Reason: Borderline Personality Disorder is characterized by a pervasive pattern of instability in relationships, self-image, and affect, and marked impulsivity. This includes poor self-esteem, intense fear of abandonment (dependency needs), and impulsive behaviors.

38. Answer: a. Limit setting and flexibility in schedule

Reason: Care for a client with BPD should be structured with consistent, firm limits to provide a safe framework, but also allow for some flexibility within the structure to help the client feel a sense of control and reduce feelings of being trapped or controlled.

39. Answer: c. Generativity vs. Stagnation

Reason: According to Erik Erikson, the developmental stage for middle adulthood (ages 40-65) is Generativity vs. Stagnation. At 42 years old, this is the appropriate stage for the client.

40. Answer: b. to deal with the feelings and thoughts that are not acceptable

Reason: Projection is a primitive defense mechanism where a person attributes their own unacceptable, anxious, or negative feelings, thoughts, or impulses to another person. It is a way to deal with these feelings by externalizing them.

41. Answer: c. "I don't know anything about that. You are afraid of being harmed"

Reason: This response acknowledges the client's feeling (fear) without validating the delusion (that the NBI is actually out to get him). It is therapeutic to not argue with the delusion (options A & B) and to focus on the underlying emotion. Asking "What made you think that?" can reinforce the delusion.

42. Answer: b. Pseudoparkinsonism

Reason: Haloperidol (Haldol) is a typical antipsychotic. Pseudoparkinsonism is a common extrapyramidal side effect caused by these medications. Its symptoms mimic Parkinson's disease, including tremors (often pill-rolling), rigidity, and bradykinesia.

43. Answer: b. Transference

Reason: Transference occurs when a client unconsciously displaces feelings (often from childhood) onto the nurse or therapist. The client is exhibiting hostility towards the staff member that is "for no apparent reason," suggesting the feeling is being transferred from another significant person in the client's life.

44. Answer: b. Adventitious

Reason: A crisis can be categorized as:

Situational: Arising from an external, unexpected event (e.g., job loss, divorce).

Adventitious (Disaster): Arising from a natural disaster, accident, or crime. Rape is a criminal act, thus an adventitious crisis.

Developmental: Occurring during a normal life transition.

45. Answer: b. Touch the client to show empathy and acceptance

Reason: A rape victim may perceive touch as a threat and invasive, especially immediately after the trauma. The nurse should ask for permission before touching. Assuring privacy, accompanying the client, and maintaining a non-judgmental approach are all appropriate.

46. Answer: c. She postpones the physical assessment until the client is calm

Reason: Acting as an advocate means protecting the client's rights and dignity. Postponing the physical exam until the client is emotionally ready and calm respects the client's psychological state and is a key part of advocacy.

47. Answer: d. Help her return to her pre-rape level function

Reason: The overall goal of crisis intervention is to help the individual return to their pre-crisis level of functioning. It is a short-term, problem-solving therapy.

48. Answer: d. Post traumatic disorder

Reason: Five months after a traumatic event, the client is experiencing symptoms of re-experiencing (difficulty concentrating can be a sign of hyperarousal), avoidance (guilt can be a way to avoid the memory), and neurovegetative symptoms (poor appetite, insomnia), which are

characteristic of Post-Traumatic Stress Disorder (PTSD).

49. Answer: c. Establishing one's sense of competence in school

Reason: A 14-year-old is in the adolescent stage. According to Erikson, the primary psychosocial task is Identity vs. Role Confusion, which involves establishing a sense of self in relation to peers and school. However, the question asks for the developmental focus in this age group based on traditional developmental theories, which often emphasizes developing a sense of competence in school and social roles. This aligns with Erikson's Industry (school-age) leading into the exploration of identity.

50. Answer: d. Passive

Reason: The client's somatic complaint (bronchial asthma) appears to be a response to emotional stress (mother leaving). Somatic complaints without an identifiable organic cause are often associated with a passive personality type, where the individual expresses emotions internally rather than verbally.

51. Answer: c. A living, learning and working environment

Reason: A therapeutic milieu (or milieu therapy) is a structured and supportive environment that provides a safe space for clients to learn, socialize, and work on their therapeutic goals. It is a "living, learning, and working" environment.

52. Answer: c. Place in semi-Fowler's position and render O₂ inhalation as ordered

Reason: The client is admitted for bronchial asthma. The priority is to address the physiological airway, breathing, and circulation (ABC) needs. Placing the client in a semi-Fowler's position and providing oxygen is the highest priority to support respiratory function.

53. Answer: c. "You seem to have concerns about going home"

Reason: This is a therapeutic response that uses the technique of validating or making an observation. It acknowledges the client's implied feelings and opens the conversation for the client to express their specific concerns without making assumptions or giving false reassurance.

54. Answer: c. Comfort with one's sexuality

Reason: A nurse's ability to discuss sexuality with a client is most fundamentally influenced by their own comfort level and attitudes towards their own sexuality. Without this, it is difficult to be therapeutic and non-judgmental.

55. Answer: d. It is the desire to live or involve in reactions of the opposite sex

Reason: Gender Identity Disorder (now often referred to as Gender Dysphoria) is characterized by a strong and persistent identification with the opposite sex and a persistent discomfort with one's own sex. The other options describe paraphilias.

56. Answer: b. sexual arousal

Reason: The sexual response cycle has four phases: Excitement (or Arousal) → Plateau → Orgasm → Resolution. The arousal phase is where the sexual interest or excitement continues to build.

57. Answer: b. Sexual Arousal Disorder

Reason: The question asks for the inability to maintain the physiological requirements for intercourse. This is the definition of Sexual Arousal Disorder (e.g., inability to attain or maintain an erection in men, or adequate lubrication in women). Sexual Desire Disorder is about a lack of interest or desire.

58. Answer: d. "I only need access to your arm, putting up your sleeve is fine"

Reason: The client's comment is sexually suggestive. The nurse's response should be therapeutic, setting a clear and firm boundary without being punitive or personal. It refocuses the interaction on the task at hand (taking blood pressure) without encouraging the behavior.

59. Answer: b. fluid volume deficit

Reason: The client was admitted due to "weight loss and dehydration secondary to starvation." In Maslow's hierarchy of needs and the ABCs of nursing priorities, a fluid volume deficit is a life-threatening physiological need that must be addressed before other issues like self-image or family processes.

60. Answer: c. Approach the nurse and talk out her feelings

Reason: For a client with anorexia nervosa, the impulse to starve is often a maladaptive coping mechanism to deal with anxiety and feelings of lack of control. The best intervention is to help the client learn to verbalize those feelings instead of acting on them through starvation.

61. Answer: b. gain weight

Reason: While a more realistic self-concept and participation in activities are positive signs, the primary and most definitive indicator of improvement in anorexia nervosa is consistent and progressive weight gain towards a target goal.

62. Answer: a. have episodic binge eating and purging

Reason: Bulimia nervosa is specifically characterized by recurrent episodes of binge eating followed by inappropriate compensatory behaviors (purging) to prevent weight gain. While anorexics may also purge, it is the binge-eating cycle that is the hallmark of bulimia.

63. Answer: a. Patient will learn problem solving skills

Reason: The diagnosis is "powerlessness related to feeling not in control." The most direct goal is to help the patient gain a sense of control by learning effective problem-solving skills to manage their triggers and feelings instead of using bingeing and purging as a way to cope.

64. Answer: d. Teach patient about bulimia nervosa

Reason: This is a component of the orientation phase and is important, but the question asks what promotes the therapeutic relationship itself. Establishing trust, discussing the eating behavior without judgment, and helping the patient identify feelings are all direct actions that build rapport and the therapeutic alliance.

65. Answer: c. Claustrophobia

Reason: Claustrophobia is an intense fear of enclosed spaces. An elevator is a classic example of an enclosed space that triggers this phobia.

66. Answer: c. Establish trust through a consistent approach

Reason: As with any therapeutic relationship, especially with a client experiencing significant anxiety, establishing trust is the first and most crucial intervention. A consistent, reliable approach from the nurse will help the client feel safe.

67. Answer: a. revealing personal information to the client

Reason: Countertransference occurs when the nurse unconsciously displaces feelings or personal issues onto the client. Revealing personal information to a client is a sign of countertransference (the nurse is blurring the

professional boundary). Option D is the definition of transference (client's feelings towards the nurse).

68. Answer: d. The client will be able to overcome his disabling fear

Reason: Systematic desensitization is a behavioral therapy technique used to reduce phobias and anxiety. The ultimate desired outcome is for the client to successfully confront and overcome their specific fear.

69. Answer: a. Avoid taking CNS depressant like alcohol.

Reason: Valium (diazepam) is a benzodiazepine and a central nervous system (CNS) depressant. Combining it with other CNS depressants like alcohol can lead to severe respiratory depression, excessive sedation, and even death.

70. Answer: c. The conversion symptom has symbolic meaning to the client

Reason: In conversion disorder, the physical symptom (e.g., paralysis) is not under voluntary control and serves a psychological purpose, often symbolizing an underlying conflict or providing a solution to an intolerable situation (primary gain). The anxiety is typically decreased, not increased (B), and is unconscious, not conscious (A).

71. Answer: d. "How do you feel about being pressured into sex by your boyfriend?"

Reason: This is the most therapeutic response as it is open-ended, explores the client's feelings without judgment, and encourages verbalization. It does not make assumptions (A, C) or give advice (B).

72. Answer: b. It is a deliberate effort to handle upsetting events.

Reason: Malingering is the deliberate fabrication or exaggeration of physical or psychological

symptoms for external gain (e.g., avoiding work, obtaining money). Somatoform disorders are unconscious expressions of psychological distress.

73. Answer: d. Psychotherapy

Reason: Since malingering is a deliberate behavior, it is best managed by addressing the underlying motivation and environmental triggers, often through psychotherapy or confrontation. It is not a physiological disorder and won't respond to a medical regimen or stress management in the same way a psychophysiological disorder would.

74. Answer: c. He learns to verbalize his feelings and concerns

Reason: The long-term goal for somatoform/conversion disorders is for the client to learn to identify and express their feelings and conflicts verbally, rather than converting them into physical symptoms.

75. Answer: b. "I haven't been able to open the door and go into my baby's room"

Reason: Dysfunctional grieving involves being stuck in the grieving process. Actively avoiding reminders of the deceased (the child's room) a year after the death indicates a maladaptive avoidance pattern, supporting the diagnosis.

76. Answer: c. Low esteem related to failure in role performance.

Reason: The client's statement "I'm good for nothing" directly reflects a negative self-evaluation (low self-esteem) that is specifically tied to her perceived failure in her role as a mother ("can't even take care of my baby").

77. Answer: b. Monitor respiratory status

Reason: While confusion, reorientation, and documentation are important, the highest priority immediately post-ECT is monitoring the

client's airway, breathing, and circulation (ABCs) due to the effects of the general anesthetic and muscle relaxants used during the procedure.

78. Answer: a. What is causing you to become agitated?

Reason: When a client is agitated but not yet violent, the nurse should assess the situation by asking open-ended questions. This helps the client identify the trigger and gives them an opportunity to problem-solve before losing control.

79. Answer: d. Initiate confinement measures

Reason: When the client's anger is escalating, the nurse should use de-escalation techniques (acknowledging behavior, maintaining distance, moving to a quiet area). Initiating confinement measures (seclusion/restraints) is a last resort when all other less-restrictive measures have failed, and the client is a danger to self or others. It is not the first or least helpful approach at this stage.

80. Answer: b. A mature experienced nurse

Reason: A client with potential for violence requires a nurse who is calm, confident, experienced, and able to maintain therapeutic boundaries and control. An inexperienced or timid nurse is at higher risk of being manipulated or injured.

81. Answer: a. Taking a directive role in verbalizing feelings

Reason: This is a rights-respecting intervention. The nurse is helping the client to express their anger verbally, offering them control and a choice, rather than immediately resorting to coercive measures like seclusion or restraints.

82. Answer: c. The staff carried out less restrictive measures but were unsuccessful

Reason: Documenting that the least restrictive measures were attempted and failed is crucial to safeguard the client's rights. It justifies the use of restraints as a last resort and demonstrates that the staff acted ethically.

83. Answer: a. "Are you being threatened of hurt by your partner?"

Reason: When assessing for abuse, the nurse should ask direct, specific, and non-judgmental questions. The most direct and clear question about physical harm is the most appropriate. Asking what happens "when you argue" can be too vague.

84. Answer: a. Sexual Desire disorder

Reason: "Persistent distaste for sex" is the definition of a hypoactive sexual desire disorder. It is the persistent or recurrent deficiency (or absence) of sexual fantasies/desire for sexual activity.

85. Answer: a. "Here's the number of a crisis center that you can call for help"

Reason: This is the best response as it provides information and resources without giving advice (B), judging (D), or assuming she hasn't spoken to family (C). It empowers the client to make her own decisions while offering support.

86. Answer: b. "When I tell my child to do something once, I don't expect to have to tell"

Reason: This expectation is developmentally inappropriate for a 3-year-old, who often needs repetition. It indicates a high level of frustration and unrealistic standards from the parent, which is a risk factor for abuse.

87. Answer: c. Ensure the safety of the victim

Reason: The nurse's primary legal and ethical responsibility in cases of suspected or

confirmed child abuse is to ensure the immediate safety of the child victim.

88. Answer: c. Severe

Reason: The client is exhibiting pacing, rapid speech, headache, and inability to focus. These are signs of severe anxiety, where the perceptual field is greatly narrowed and the client has difficulty focusing on details.

89. Answer: b. a subjectively perceived threat

Reason: Anxiety is a diffuse, unpleasant, and vague sense of apprehension that is triggered by a perceived (subjective) threat to one's self-concept, values, or safety. Fear is a response to an objective, known threat.

90. Answer: a. Give specific instructions using speak in concise statement.

Reason: With severe anxiety, a client's ability to process complex information is impaired. The nurse should use short, simple, concise, and concrete statements. Asking to "identify the cause" or explaining details (B, C) would be overwhelming. Urging them to focus (D) is not as effective as providing simple directions.

91. Answer: b. Valium

Reason: Valium is the medication of choice for acute, severe anxiety. Prozac is an antidepressant, Risperdal is an antipsychotic, and Lithium is a mood stabilizer. (Note: There's a numbering error in the original document where 791 appears after 90).

92. Answer: b. Take medication after meals

Reason: Valium can cause gastric irritation. Taking it after meals can help minimize this side effect. Option A is a dietary instruction for MAOIs, C is false (must be tapered), and D is incorrect (should never double up on medication).

93. Answer: b. "Discussing suicide with a client is not harmful"

Reason: This is a fundamental concept in suicide prevention. Discussing suicide with a client does not plant the idea in their head. It is therapeutic and actually reduces anxiety.

94. Answer: c. A 75 year old male client with cancer

Reason: The highest risk factors for suicide are being an older, white male, often with a chronic or terminal physical illness (like cancer) and social isolation.

95. Answer: d. Abuse occurs more often in low-income families

Reason: Abuse occurs across all socioeconomic, racial, and ethnic groups. It is a myth that it only happens in low-income families. The other statements are true characteristics of abusers.

96. Answer: a. Peppermint pizza, salad and cocoa cola

Reason: The question asks for a safe diet for a client on an MAOI (Parnate). A tyramine-restricted diet is required. Peppermint pizza (no pepperoni/sausage), salad, and cola do not contain high levels of tyramine. The other options contain tyramine-rich foods (roasted chicken/beer can be an issue, pickled herring is very high).

97. Answer: c. Check with the psychiatrist before using any over the counter medications

Reason: Many over-the-counter (OTC) medications (especially NSAIDs) can interact with lithium and increase serum levels, leading to toxicity. This is the most important safety teaching point.

98. Answer: d. Hold the 4:00 PM dose and notify the physician of the client's complaints.

Reason: The client is exhibiting signs of lithium toxicity (nausea, drowsiness, muscle weakness, lack of coordination). The nurse should hold the dose and notify the physician immediately to get a stat lithium level and prevent further toxicity.

99. Answer: c. "Taking my fluoxetine (Prozac) and a Xanax before I leave have helped me to cross the bridge and go to work every morning."

Reason: This statement shows the client is successfully using the prescribed pharmacological (Prozac and Xanax) and behavioral interventions to function (cross a bridge and go to work), indicating a good response to treatment.

100. Answer: d. Instructing the client hyperventilate to create panic attack.

Reason: Paradoxical intention is a technique where the client is instructed to deliberately bring on the feared symptom (e.g., hyperventilate to create a panic attack). This breaks the cycle of "fear of the fear" and shows the client they can control the symptoms.

101. Answer: b. "It is scary when you feel out of control with such feelings of emptiness and anger that you can't stop yourself."

Reason: This response validates the client's feelings of being out of control and provides empathy. It doesn't judge, lecture, or give simplistic advice, which is appropriate for a client with borderline personality disorder.

102. Answer: b. manipulative behavior

Reason: Antisocial personality disorder is characterized by deceitfulness, manipulation, and exploitation of others for personal gain.

103. Answer: d. "I'll stay here with you until you get better."

Reason: This response demonstrates acceptance, support, and therapeutic presence. It does not make demands ("Get off the bed") or give vague offers of help ("Just call me"). It is a gentle invitation to connect.

104. Answer: d. dysphoria

Reason: Narcissistic personality disorder is characterized by grandiosity, arrogance, and hypersensitivity to criticism. Dysphoria (a state of unease or dissatisfaction) is not a defining trait; in fact, they often present with a sense of entitlement and superiority.

105. Answer: b. Help identify patient's strengths

Reason: Clients with avoidant personality disorder have very low self-esteem and a fear of rejection. Interventions should focus on building their self-confidence by identifying and reinforcing their strengths in a safe, non-threatening environment.

106. Answer: b. fatigue

Reason: Mania is characterized by a distinct period of abnormally elevated, expansive, or irritable mood, with increased energy. Symptoms include distractibility, irritability, and a decreased need for sleep (insomnia). Fatigue is not a characteristic symptom of a manic episode.

107. Answer: c. paranoid behavior

Reason: Erikson's first stage is Trust vs. Mistrust (Infancy). Failure to master this stage leads to a fundamental sense of mistrust in the world and others. A lack of trust can manifest later as suspiciousness, mistrust, and paranoid behaviors.

108. Answer: c. lack of guilt

Reason: Manipulation and aggression in conduct disorder are often driven by a lack of empathy, remorse, and guilt, which allows the child to violate others' rights without feeling bad about it.

109. Answer: a. early mother and child separation

Reason: Psychosocial theorists suggest that schizoid personality disorder, characterized by emotional detachment, is linked to an early childhood history of maternal deprivation or a cold, unresponsive family environment, often with inadequate nurturing.

110. Answer: d. Somatoform pain disorder

Reason: The client is complaining of pain (low back pain) that is severe enough to cause significant distress and functional impairment (absences from work), but there is no organic basis found for it. This fits the diagnosis of pain disorder (historically known as somatoform pain disorder).

111. Answer: b. Tolerance

Reason: Tolerance is the need for markedly increased amounts of a substance to achieve the desired effect (or a markedly diminished effect with continued use of the same amount). This is a physiological adaptation to the drug.

112. Answer: a. Delirium tremens

Reason: These symptoms (increased tremors, irritability, hypertension, fever) are classic signs of alcohol withdrawal, which can progress to Delirium Tremens (DTs), a medical emergency characterized by severe autonomic hyperactivity, hallucinations, and confusion.

113. Answer: a. Monitoring his vital signs every hour

Reason: During detoxification, the client is at risk for severe withdrawal symptoms (as seen in

Q112). The priority is to monitor vital signs frequently to detect cardiovascular instability and signs of impending DTs, ensuring client safety.

114. Answer: b. Cocaine

Reason: Cocaine use is associated with symptoms of agitation, hallucinations, and tachycardia. A key physical sign is an irritated nasal septum, which is caused by chronic snorting of the drug.

115. Answer: b. Narcan (Naloxone)

Reason: The client has symptoms of opioid overdose (needle tracks, stupor, pinpoint pupils). Naloxone (Narcan) is an opioid antagonist used as an emergency treatment to reverse the effects of opioid overdose.